

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 77 yrs | Sex: Male | MRN: SMC-04471932
Encounter Date: January 17, 2023 | Provider: Laura Simmons, MD — Emergency Medicine | Doc ID: SMC-2023-0117-ED

Emergency Department Visit

Mechanical Fall with Left Distal Radius Fracture

HPI

77-year-old man slipped on ice and fell onto outstretched left hand. Complains of left wrist pain and deformity. No head strike, no loss of consciousness, no preceding dizziness/palpitations (no syncope). On apixaban — assessed for occult bleeding.

Vital Signs

BP (mmHg)	HR (bpm)	RR	Temp	SpO2	Weight	BMI
138/82	74 irreg	16	36.6 C	98% RA	90.5 kg	29.5

Examination

Left wrist swollen with dorsal 'dinner-fork' deformity, tender; neurovascularly intact (median/ulnar/radial sensation normal, capillary refill brisk). No head/neck injury. No other deformity. Gait not tested due to wrist.

Imaging

Left wrist radiographs: dorsally angulated, extra-articular distal radius fracture (Colles-type) with mild dorsal comminution; ulnar styloid intact. CT head (given anticoagulation + fall): no intracranial hemorrhage.

ED Course & Plan

- Closed reduction performed under hematoma block (lidocaine); satisfactory alignment achieved; sugar-tong splint applied; post-reduction films acceptable.
- Orthopaedics follow-up in 1 week for casting vs operative discussion.
 - Continue apixaban (no intracranial bleed).
 - Given fragility fracture from standing height: order DEXA scan and bone-health evaluation (see bone-health note).
 - Analgesia: acetaminophen (avoid NSAIDs — CKD); short opioid course if needed.
 - Falls assessment recommended.

Medications This Visit

Medication	Dose	Route	Frequency	Indication
Lidocaine 1%	10 mL	Local	Once	Hematoma block for reduction
Acetaminophen	1000 mg	PO	Every 8 h PRN	Analgesia